SAMPLE ANSWER · STROKE · CASE 3

Sample 7-minute ISBAR: Leilani

Leilani Fonoti · 63-year-old woman · Community occupational therapy home visit · about 1023 spoken words (≈ 7.3 min at 140 words per minute)

I Introduction

59 words

"Hi, I'm the occupational therapy student. I'm calling you from Leilani Fonoti's home, where I came to check her shower equipment. Leilani is 63 and had a right hemisphere stroke; she was discharged from rehab four weeks ago. I need your help now, because I'm worried about her immediate safety and her husband's ability to keep caring for her."

S Situation

130 words

"The equipment is fine, but Leilani is very drowsy. She drifted off mid-sentence twice and only woke when Tavita said her name. She has had three falls this week, and she has a large bruise on her left forearm and a fresh, uncovered skin tear on her left shin.

Tavita, her husband and main carer, told me privately that he can't keep doing this. He hurt his back lifting her off the bathroom floor on Tuesday, and he has been giving her one or two of his own prescribed sleeping tablets at night, the last at about 2 am. He's planning to give her half a tablet after lunch and then lie down himself. My next visit is due in 20 minutes, but I don't think I should leave yet."

B Background

153 words

"Leilani had a right hemisphere stroke with left arm and leg weakness. At discharge four weeks ago, she needed standby assistance from one person to transfer and walked short distances indoors with a quad stick and supervision. Today Tavita took most of her weight, which is a clear change.

She lives with Tavita, who is 68. Their daughter Mele lives in Sydney. Leilani worked as a teacher aide and has sung in her church choir for more than 30 years. Returning to the choir is her goal.

There's a lot I don't know. I don't have a list of Leilani's own medications, including whether she takes blood thinners. I don't know whether she hit her head in any of the falls, or whether her GP knows about them. There's no record of community supports, respite or a carer assessment since discharge. I also don't know the name or dose of Tavita's sleeping tablets."

A Assessment

308 words

"My most urgent concern is Leilani's drowsiness. She has been given a sedative that wasn't prescribed for her, in addition to whatever she normally takes, and she has had three falls this week without anyone knowing whether she hit her head. Several possible explanations, including medication effects, a head injury or a change related to her stroke, would need urgent medical assessment, and I can't tell which it is. Sedation also increases her falls risk, and Tavita plans to give her more this afternoon.

The second concern is that Tavita seems to be at breaking point. He's exhausted, has injured his back and is lifting her alone, including off the floor. Giving his own medication to Leilani

appears to come from desperation rather than an intention to harm. But it is still a serious risk to her, and it needs to be handled through our service's safeguarding procedure, with compassion and without blame.

Third, Leilani's function has dropped. She needs much more help to transfer than at discharge, which may be partly the sedation. The current care arrangement isn't safe for either of them: every transfer risks another fall for her and further injury for him.

I want to be clear about the distractors. The equipment is installed correctly, but good equipment doesn't make the situation safe. Leilani's choir goal, Mele's NDIS question and the parking permit letter are all legitimate. I want to come back to them, but none of them should come before today's risks.

So my priorities are: first, urgent medical review of Leilani's drowsiness, and no more tablets; second, immediate support for Tavita and a safe care plan for tonight; and later, reassessment of her function, equipment and longer-term supports. I'm not making judgements about anyone's capacity or the cause of her drowsiness, but I think this needs action before I leave the house."

R Recommendation

373 words

"First, I'd stay with Leilani and keep her sitting safely. I'd gently ask Tavita not to give her any more of his tablets and to keep the box aside until a doctor has reviewed her. I'd explain that this is about her safety, not about blaming him. If she becomes harder to wake, her breathing changes or she vomits, I would call 000 straight away.

Second, I'd like your help to arrange urgent medical review now. Because she is this drowsy after falls with a possible head strike, I think that means calling an ambulance rather than waiting for a GP appointment. I'd pass on the details: what was given, when, the falls and her drowsiness. I'd also ask you to reschedule my next visit so I'm not rushing this.

Third, I'd like your guidance to follow our adult safeguarding procedure and to document exactly what Tavita told me, in his words.

For the wider team, I'd ask the GP or a pharmacist to review all of Leilani's medications and Tavita's, since he's also unwell. Community nursing could dress the skin tear. Social work could help with emergency respite through Carer Gateway and help Mele with the NDIS application, as Leilani is under 65. I'd also suggest Tavita sees his own GP about his back. If home isn't safe tonight, the team may need to consider a hospital admission or respite.

For OT, once Leilani is medically reviewed, the principal short-term assessment would be observed performance of her transfers and showering routine with Tavita assisting. That would show what assistance she really needs now and whether Tavita can safely provide it. It might mean extra equipment, a falls plan that rules out lifting her off the floor, and a personal alarm. For the longer term, I'd use the Canadian Occupational Performance Measure with Leilani, with Tavita and Mele included if she wishes. That keeps her goals, like singing in the choir and church life, at the centre. I'd also ask Leilani and Tavita who in their family or church community they would want to help.

The Stroke Foundation Living Clinical Guidelines recognise carers' needs for support and training after stroke. After today, I'd follow up within the week, including the parking permit request."

Coaching notes *not spoken*

The immediate priority

Leilani is drowsy after being given sedatives that weren't prescribed for her, and she has had three falls with possible head strike. She needs urgent medical review, and the plan to give more tablets must stop. Tavita is at breaking point and has injured himself, so the care situation is unsafe tonight.

Distractors in this case

- **Equipment correctly installed** — the task you came for is done, but it doesn't mean she is safe.
- **"She's just sleepy"** — reported as a good night's rest, but likely sedation; don't accept it.
- **Choir goal, NDIS question, parking permit** — all valid and client-centred, but they come after today's safety actions.
- **Next visit in 20 minutes** — reschedule rather than leave a risk unaddressed.

Missing information to clarify

- Leilani's current medications (including any antithrombotics) and the name and dose of Tavita's tablets
- Whether she hit her head in any fall; whether her GP knows
- Any supports, respite or carer assessment since discharge
- Whether Tavita's back injury has been assessed

Useful references

- Stroke Foundation, *Living Clinical Guidelines for Stroke Management* (carer support and training)
- Carer Gateway (Australian Government) — emergency and planned respite for carers
- Your service's adult safeguarding or elder abuse procedure

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